

GCMWC

GLOBAL COMMISSION FOR MENTAL WELLNESS COACHING

UNIVERSITY IMPLEMENTATION KIT

For colleges, universities, continuing education departments, workforce programs, churches, ministry schools, and institutional partners

Purpose of this kit

This implementation kit gives institutional leaders a clear model for adopting the GCMWC Mental Wellness Coaching curriculum as a non-clinical training program. It explains why the program matters, how to launch it, what infrastructure is needed, and how to protect scope, students, and the public.

Area	Details
Prepared for	Institutional review and pilot planning
Program	GCMWC Mental Wellness Coach Certification
Format	Online, hybrid, in-person, or LMS-supported institutional delivery
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1. Executive Overview

The Global Commission for Mental Wellness Coaching (GCMWC) helps institutions implement a structured, non-clinical mental wellness coaching curriculum with clear standards, ethical boundaries, referral expectations, and practical coaching application.

This kit is designed for leaders who need more than a program description. It gives decision-makers a clear view of the program value, adoption options, operational requirements, student safety standards, and implementation pathway.

Core position

Mental Wellness Coaching supports growth, self-awareness, wellness planning, emotional regulation, and behavior change. It does not diagnose, treat, provide psychotherapy, manage crisis care, or replace licensed medical or mental health professionals.

The institutional value is straightforward: GCMWC gives schools and organizations a way to train responsible helpers without pretending coaching is clinical treatment. That distinction matters because students, staff, families, faith communities, and workplaces are carrying more mental and emotional pressure than most systems were built to absorb.

2. Why Institutions Need Mental Wellness Coaching

Mental health needs are rising faster than most institutions can respond. Counseling centers are overloaded. Faculty and student affairs teams are encountering emotional distress. Churches and ministry schools are carrying pastoral care demands. Workforce and healthcare-adjacent programs are being asked to prepare people for human-centered support roles.

The weak spot is not compassion. The weak spot is structure. When helpers are compassionate but untrained, support can slide into advice-giving, spiritual pressure, dependency, missed red flags, role confusion, or unqualified counseling.

GCMWC helps institutions create a safer middle lane: trained non-clinical support that knows its role, knows its limits, and knows when to refer.

The pressure is real	The risk is real	The solution needs structure
Students and communities need support for stress, burnout, grief, identity, transitions, emotional regulation, and relational patterns.	Untrained helpers can cross into clinical territory without realizing it. That creates ethical, legal, and reputational risk.	Institutions need a standards-based curriculum, assessment process, referral rules, and clear scope language.

3. Institutional Fit and Use Cases

Universities and Colleges Offer GCMWC as an elective, certificate track, continuing education course, student leadership training, or wellness coaching pathway.	Community Colleges Prepare learners for wellness coaching, peer support, community support, lifestyle coaching, and human services-adjacent roles.	Workforce Programs Add a practical credential pathway for adult learners entering the wellness, community care, ministry, or support services workforce.
Churches and Ministry Schools Train ministry leaders, lay counselors, women's ministry leaders, youth workers, and pastoral care teams in ethical support.	Healthcare-Adjacent Programs Equip nurses, wellness staff, fitness professionals, health educators, and care coordinators with coaching boundaries and tools.	Nonprofits and Community Programs Train community-facing leaders serving populations impacted by stress, trauma exposure, grief, family instability, or major life transition.

4. What the Institution Receives

GCMWC provides a structured curriculum and implementation model that helps institutions launch responsibly, train consistently, and document outcomes.

Area	Details
Course-ready curriculum	A structured training framework that can be adapted for academic credit, non-credit continuing education, certificate programs, workshops, or cohort-based delivery.
Scope and ethics framework	Clear language defining what Mental Wellness Coaches can and cannot do, including referral triggers, boundary rules, and non-clinical practice standards.
Instructor guidance	Teaching notes, facilitation guidance, module objectives, discussion prompts, coaching lab structure, and assignment suggestions.
Assessment tools	Rubrics, case study assignments, coaching practice evaluations, competency checks, completion requirements, and feedback templates.
Student protection policies	Support for informed consent, confidentiality, attendance expectations, grievance procedures, completion standards, and conduct expectations.
Implementation roadmap	A pilot-ready launch plan for cohort size, delivery format, faculty roles, technology, assessment flow, and student communication.
Credential pathway alignment	Students may train toward GCMWC-aligned completion, MWC-T readiness, or MWC-C pathway requirements depending on the adoption agreement.
Marketing and enrollment support	Program descriptions, enrollment language, compliance wording, and student-facing explanations that avoid misleading claims.

5. Student Learning Outcomes

Students are trained to provide structured, ethical, non-clinical support for mental wellness. The goal is not to turn students into therapists. The goal is to equip them with practical coaching skills, referral awareness, and whole-person wellness frameworks they can use responsibly.

- Distinguish coaching, therapy, counseling, consulting, mentorship, pastoral care, and peer support.
- Use active listening, reflective questioning, values clarification, and goal-setting to support client growth.
- Recognize distress signals and referral needs without diagnosing or treating mental health conditions.
- Apply non-clinical tools such as journaling, grounding, habit tracking, wellness planning, and accountability.
- Integrate mental, emotional, physical, relational, and spiritual wellness concepts in a responsible way.
- Discuss faith, values, and spirituality ethically without manipulation, coercion, or theological overreach.
- Understand how lifestyle, nutrition, sleep, stress, and behavior patterns influence whole-person wellness.
- Document coaching practice, communicate boundaries clearly, and refer when needs exceed coaching scope.
- Build ethical coaching services, support roles, or community care initiatives with accurate marketing language.

6. Curriculum Overview

The curriculum is modular so institutions can align it with academic calendars, cohort delivery, continuing education units, ministry school schedules, or workforce training models.

Area	Details
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Module 1: Foundations	Coaching roles, agreements, ethical boundaries, active listening, powerful questions, client autonomy, and coaching vs. therapy.
Module 2: Mental Wellness Frameworks	Stress, emotional regulation, burnout, trauma-informed awareness, red flags, referral protocols, and non-clinical support strategies.
Module 3: Faith-Based Integration	Ethical integration of faith, prayer, identity, forgiveness, discernment, spiritual support, and non-coercive practice.
Module 4: Nutrition and Emotional Health	Gut-brain connection, hydration, blood sugar, mood-supportive habits, food patterns, and referral to licensed professionals when needed.
Module 5: Pharmacology Awareness	Basic awareness of medications, supplements, safety concerns, contraindications, collaboration, and strict limits of coaching scope.
Module 6: Wellness Coaching Strategies	Behavior change, motivational interviewing concepts, emotional intelligence, action planning, habit formation, accountability, and client-centered support.
Module 7: Practice Development	Coaching agreements, documentation, ethical marketing, pricing, client communication, case studies, observed coaching, and capstone application.

7. Adoption and Delivery Models

Pilot Cohort Model Best for schools that want to test the curriculum before full adoption. A small cohort completes the program over 8 to 12 weeks while the institution tracks engagement and outcomes.	Continuing Education Certificate Best for professional learners and adult education. The institution offers the program as a non-credit certificate for professionals seeking coaching skills.	Academic Course Integration Best for colleges and universities. Curriculum can be adapted into an elective, certificate course, practicum, health education course, or leadership course.
Institutional Training Partner Best for organizations that want to deliver GCMWC-aligned training consistently under an adoption agreement.	Church or Ministry Institute Track Best for churches and ministry schools that want ethical pastoral support training without crossing into counseling or therapy.	Workforce Development Track Best for community programs preparing adult learners for wellness, peer support, and community care roles.

8. What It Takes to Launch

Institutions do not need to build a mental wellness coaching curriculum from scratch. They need the right structure, faculty alignment, student support process, and administrative clarity.

Area	Details
Program Lead	Designated institutional contact responsible for scheduling, communication, student questions, implementation decisions, and GCMWC coordination.
Qualified Instructors	Faculty, trainers, or approved facilitators who understand coaching ethics, mental wellness boundaries, referral standards, and non-clinical scope.
Delivery Platform	LearnWorlds, Canvas, Moodle, Teachable, Thinkific, Google

	Classroom, or the institution's internal LMS.
Live Training Capacity	Zoom, Google Meet, in-person classroom space, or hybrid meeting structure for coaching labs, role-play, peer coaching, and observed practice.
Assessment Process	System for collecting assignments, scoring rubrics, reviewing coaching demonstrations, tracking completion, and documenting feedback.
Student Support	Onboarding, attendance expectations, technology guidance, grievance procedure, confidentiality rules, and referral language.
Compliance Language	Public-facing language must state that Mental Wellness Coaching is non-clinical and does not authorize diagnosis, psychotherapy, medical care, or crisis intervention.

9. Sample 10-Week University Format

Area	Details
Week 1: Orientation and Foundations	Coaching identity, scope, ethics, agreements, confidentiality, and the difference between coaching and clinical care.
Weeks 2-3: Mental Wellness Frameworks	Stress, burnout, emotional regulation, trauma-informed awareness, behavior patterns, red flags, and referral standards.
Week 4: Faith and Values Integration	Supporting spirituality, values, prayer, purpose, and identity without coercion or theological overreach.
Week 5: Lifestyle and Emotional Health	Nutrition, hydration, sleep, movement, routines, and emotional wellness within a non-clinical education model.
Week 6: Pharmacology and Referral Awareness	Basic medication and supplement awareness, safety considerations, and when coaching must pause for medical referral.
Weeks 7-8: Coaching Strategy Labs	Active listening, questions, behavior change tools, goal setting, motivational interviewing concepts, and accountability.
Week 9: Professional Practice	Coaching agreements, documentation, client boundaries, pricing, marketing claims, and role clarity.
Week 10: Observed Coaching and Capstone	Practical coaching demonstration, case reflection, capstone plan, and final evaluation.

10. Scope, Safety, and Referral Protocol

Institutional adoption must protect students, clients, and the public. GCMWC curriculum includes boundaries around scope of practice, referral triggers, ethics, confidentiality, and documentation.

Required disclaimer

This program provides coach training and education. It does not license participants to diagnose, treat, provide psychotherapy, prescribe, manage medication, provide crisis intervention, or replace licensed medical or mental health care.

Referral is required when a student or coaching participant presents concerns that exceed the coaching scope, including:

- Suicidal ideation or homicidal ideation

- Psychosis, hallucinations, or severe disorganization
- Active addiction preventing meaningful engagement
- Severe depression or functional decline
- Domestic violence, abuse, neglect, or exploitation
- Eating disorder symptoms
- Medical instability
- Need for diagnosis, treatment, medication management, or crisis intervention

Institutional safety actions should include pausing the coaching process, naming the boundary calmly, encouraging referral to appropriate services, documenting the referral recommendation, and avoiding continued coaching as a substitute for needed clinical care.

11. Faculty, Technology, and Operations

Area	Details
Faculty preparation	Faculty should be oriented to GCMWC scope, ethics, referral triggers, coaching lab facilitation, assessment rubrics, and documentation expectations.
Recommended ratio	For live coaching labs, use small groups or triads. A practical target is no more than 20 students per primary instructor, with additional facilitators as needed.
LMS setup	Create pages for orientation, modules, assignments, rubrics, case studies, peer coaching logs, capstone instructions, and technical support.
Live session tools	Use Zoom or Google Meet with breakout rooms, attendance tracking, recordings when appropriate, and clear confidentiality instructions.
Student records	Track attendance, assignment completion, quiz scores, coaching practice hours, observed coaching feedback, capstone evaluation, and completion status.
Support workflow	Define who handles tech issues, grading questions, ethics concerns, student absences, referral escalation, and grievance submissions.

12. Assessment and Completion Standards

The program should measure both knowledge and applied coaching skill. Completion should require participation, assignments, coaching practice, case analysis, and a final evaluation.

Area	Details
Knowledge checks	Quizzes or written checks after each module to assess comprehension of scope, ethics, and coaching concepts.
Reflection journals	Short written reflections that build self-awareness, ethical discernment, and integration of course concepts.
Case studies	Written analysis of real-world scenarios requiring scope decisions, referral judgment, and coaching plan design.
Peer coaching	Structured practice sessions where students rotate as coach, coachee, and observer.
Observed coaching	Faculty or approved evaluator reviews live or recorded coaching

	against a practical skills rubric.
Capstone	Final integration project demonstrating student understanding of coaching structure, boundaries, client support, and professional practice.

13. Partnership Process

1. Discovery Call: Review your institution's goals, audience, timeline, delivery model, and desired outcomes.
2. Curriculum Fit Review: Determine how GCMWC fits your calendar, CE model, academic department, ministry school, or workforce program.
3. Implementation Plan: Define cohort size, course length, instructor roles, technology, assessment expectations, student communication, and launch timeline.
4. Faculty and Compliance Alignment: Train the delivery team on scope language, referral standards, evaluation tools, and approved use of GCMWC materials.
5. Pilot Launch: Launch a controlled pilot cohort with defined reporting, student feedback, faculty notes, and completion tracking.
6. Review and Scale: Review outcomes, refine implementation, and determine whether to expand.

14. Institutional Readiness Checklist

- ☐ We have identified the audience and institutional purpose for the course.
- ☐ We have selected the delivery model: pilot, continuing education, academic course, ministry training, workforce program, or partner track.
- ☐ We have assigned a program lead.
- ☐ We have identified qualified instructors or facilitators.
- ☐ We have selected an LMS or course delivery platform.
- ☐ We have live session capacity for coaching labs and discussion.
- ☐ We have an assessment plan for quizzes, assignments, coaching demonstrations, and capstone review.
- ☐ We have student support policies for attendance, grievance, confidentiality, conduct, and technology issues.
- ☐ We have referral language and crisis escalation procedures.
- ☐ We have reviewed public-facing marketing copy for accuracy and compliance.
- ☐ We have a pilot timeline and launch date.
- ☐ We have a plan to collect feedback and improve after the pilot.

15. Sample Institutional Copy

Short course description

The GCMWC Mental Wellness Coach Certification Program is a structured non-clinical training program that prepares learners to support mental, emotional, physical, and spiritual wellness through coaching skills, ethical boundaries, referral awareness, and practical behavior change strategies.

Catalog description

This course introduces students to the foundations of Mental Wellness Coaching, including coaching ethics, scope of practice, mental wellness frameworks, faith and values integration, lifestyle education, referral awareness, coaching strategy, and professional practice development. Students learn how to provide non-clinical support while maintaining clear boundaries between coaching and diagnosis, psychotherapy, crisis intervention, and medical care.

Student-facing disclaimer

This program provides coach training and education. Completion does not authorize diagnosis, treatment, psychotherapy, medical care, medication management, or crisis intervention. Students must follow all applicable laws and institutional policies.

Sample announcement

Our institution is reviewing a partnership with the Global Commission for Mental Wellness Coaching to offer a structured Mental Wellness Coaching curriculum. The program is designed for learners who want practical coaching skills, ethical scope boundaries, whole-person wellness education, and referral awareness. This is a non-clinical training pathway and does not replace licensed mental health or medical care.

16. Appendices and Templates

Appendix A: Pilot Application Intake Questions

- Full name
- Email and phone number
- Current role or department
- Why are you interested in Mental Wellness Coaching?
- What population do you hope to support?
- Can you attend live sessions and complete assignments?
- Do you understand this is non-clinical training?
- Do you agree to follow confidentiality and scope expectations?

Appendix B: Pilot Data to Track

- Enrollment count
- Attendance rate
- Assignment completion rate
- Quiz averages
- Observed coaching scores
- Capstone completion rate
- Student satisfaction results
- Faculty implementation notes
- Referral or safety concerns
- Recommended improvements

Appendix C: Suggested Launch Timeline

Area	Details
Weeks 1-2	Discovery, institutional approval, agreement review, audience selection, and program lead assignment.
Weeks 3-4	LMS setup, faculty orientation, assessment setup, compliance review, and student application launch.
Weeks 5-6	Enrollment confirmation, orientation, tech setup, student onboarding, and final schedule confirmation.
Weeks 7-16	Course delivery, coaching labs, assignments, assessments, feedback, and capstone.
Weeks 17-18	Completion review, feedback survey, institutional debrief, improvement log, and scale decision.

Appendix D: Contact

Global Commission for Mental Wellness Coaching

Website: www.gcmwc.org

Email: support@theperfectrx.com

Phone: 323-568-1700

Social: @gcmwc_coaching

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READY TO START A PILOT?

Start controlled. Train responsibly. Document outcomes. Scale what works.

Next step

Book a curriculum review call or request the institutional partnership packet. GCMWC will help your team determine the best adoption model, pilot size, delivery format, and compliance language before launch.